UK Data Service

Pioneers

of Qualitative Research

Sarah Arber

Life story interview with Blanche Girouard

2017

Principle investigator's thematic highlights

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Sarah Arber

Interviewed by Blanche Girouard

26 April 2017

1: Early Years: a Rebellious Path to LSE

I was born in Chingford, Essex. I'm an Essex girl! I've always been awkward throughout my life, so I came rather too quickly, and my mum said, "Oh, I think something's happening", and apparently, in the bathroom, I appeared! And so my dad and grandmother did whatever, and I was booked into a Nursing Home, so my father carried me into the Nursing Home the next day, because I'd been booked in for ten days with my mother. But I wasn't born in there, I was born in my grandmother's house in Chingford.

My parents, they were very boring. Yes, my father went to a kind of a state education, but it was like a Grammar School, but left at 16. So he was a bright lad. And then he went into the Pearl Assurance Company at the age of 16, I think, in a clerical - and then he stayed with the Pearl Assurance Company until he retired at 62-ish, worked his way up from nothing to Personnel Manager for the whole of Pearl Assurance… And my boring mother was a housewife throughout her life, and had four children.

That's quite a negative portrayal of your parents!

No, no, no, no, no. My father was a very bright able man, but I always would criticise him for having the same job his whole life. … There was no university in his family at all. He was as one of eight children. Grew up in a pub, basically. His parents ran a pub. In Haggerston Road, in the East End. So he was very much an East End lad, and they were - well, I think they made good reasonably, but they weren't educated people, or professional people…

I only just scraped through the 11+, because all my friends went to Tiffins in Kingston, and when we had to choose what Grammar School, having passed the 11+, they all went to Tiffins, and I put down Tiffins, and my 11+ school was so bad, they didn't let me to go to

Tiffins. So my parents had to look for another Grammar School who would take me. Again, I'm very thankful that they did that, and I ended up in somewhere called Sir William Perkins's School, which was a Grammar School for Girls in Chertsey. So I had a home life, and, yes, my teenage years were, I suppose, somewhat rebellious and different, and then I had the school, Grammar School life, but I didn't see the Grammar School friends at all at weekends or after school, or nothing like that – well, largely didn't, anyway.

At school, are there any key teachers who you remember, who could have been responsible for how you ended up in sociology?

No. I think I was a bit of a rebel, and I didn't get on with the Headmistress terribly well… I was just different! No, I did, I always saw myself, and I think they saw that as somewhat different. But they didn't really have aspirations, and they, as I said, they'd never been to university, and none of their relatives or anything like that had been to university, so we were very much kind of middle-class, suburban Surrey.

Did you feel different in the family?

Well, I suppose I did. I did do different things. I mean, I had a fairly rebellious youth, so I used to go to Eel Pie Island in Twickenham, which you may or may have heard of, but it was kind of jazz, and old blues and all that sort of thing, and a nice blues club, cellar club in Kingston, you know, from the age of 15, 16, so I kind of did those sorts of things…

I think we could apply for five different courses at five different universities, and I applied for five different things, one was archaeology at Cardiff, and I could have gone there… But then I decided I didn't want to do archaeology. I don't know, I've always been into archaeology and am very interested now, so I spend my life going round interesting archaeological sites. Whenever I go anywhere – I love archaeology, it's - it's, yes.

This Bridget, she actually went to Cambridge, and her mother – she was the third generation woman to go to Cambridge. She was my best friend at school – so in terms of friends she has the most, well, we perhaps influenced each other. But she was very different from me in parental heritage because her mother went to Cambridge, and her grandmother went to Cambridge as well, so, of course, she went to Cambridge, whereas the thought of me going to Cambridge - I mean, going to university was enough! And she did do archaeology, anthropology.. She knew Prince Charles because he did archaeology, anthropology… So we

did do archaeology, we were both interested in archaeology._We just liked going away and doing interesting things.

As I say, I applied for five different courses at different universities, and one was sociology at LSE. I think it was Social Anthropology I applied for at LSE, and then I applied for genetics in Birmingham, so I could have been a geneticist! So that would have been fun, because I was really interested in genetics and viruses, because I did biology and botany and zoology at 'A' Level. Anyway, but sociology - the reason for going to LSE was that it was radical. I was applying in 1967, there were the troubles, David Adelstein, all these interesting people going on there, and I thought, "I want to go somewhere interesting". So that was the main reason, was to go to LSE and it wasn't particularly sociology – I could have done anything. So it wasn't particularly sociology. (pp.1- 8)

2: Voluntary Service Overseas, Jamaica, and into Sociology

I applied to do VSO and I was interviewed, and I got accepted. As you do! And at the time they did VSO, they called them "cadets", you could go at 18, so I got trained to go to West Africa. I was supposed to go to West Africa to do VSO, teach in a nice girls' school, science subjects, and then the Biafran [war] - I got a try, I had a week's trial in Birmingham.

It was in Nigeria, that's right. A week's training for West Africa, in Birmingham – Aston University, I remember – at the grand age of 18. And then the Biafran War broke out, 1967, and so no VSOs could go to Nigeria because of the Biafran War. So I said, "Oh well!" So I went off on a dig, as you do, and I remember I was digging at the end of August or early September, it was a Bronze Age burial site in Dorset, by the coast, and I happened to ring my mother - which I did, once a week or something – and said, "Hello, Mum, how are things?" "So she said, "Oh, I'm glad you rang! We've had letters, calls, from VSO, they want you to go out in four days time to Jamaica". So, I had, "Oh, my God! I'd better leave this dig and, and come home!"

They didn't have any way of contacting me, know where I was, or e-mail or anything like that, there was no way of contacting me basically, so then four days later, I remember my parents put me on a British Airways flight to Jamaica – this was September 1967 – which went via Bermuda and New York, and Montego Bay, to get to Kingston – and I got picked up by the sports teacher, Mr. Porter, and in the car. I said, "Oh!" - this is at 18, you know, young. I said, "Oh, what am I teaching?" And he said, "Oh, we've put you in charge of the

top form", they were the GCE 'O' Level form, "and you'll be teaching geography 'O' Level". I thought, "Great! I did 'A' Level geography". "And you'll be teaching maths 'O' Level". And I said, "I can't teach maths 'O' Level". And Mr. Porter said, "Why not?" And I said, "Well, you know, I only got maths 'O' Level myself". And he said, "Well, you've got maths 'O' Level, you can teach it". And I got a B for maths, I didn't do badly, but I didn't do brilliantly. So it was a very formative one year experience…

We went to a place called Frankfield in the middle of Jamaica, in Clarendon. It was a school which had started five years ago, a big secondary comprehensive school, started five years ago…_I'd never been, I'd never - I'd been to bits of Europe, but I'd never been anywhere. I'd been on two school cruises, I'd never been abroad with my parents, so I think that was another thing is that in terms of my schooling, we had the opportunity to go on school cruises, and they were great, and very informative... I mean, that really opened my eyes to the world and culture, and archaeology and all that sort of thing – those did. So, I mean, apart from those two, I'd never been abroad, out of the country.

So how did Jamaica form you, do you think?

Well, it was fantastic, because you had to take a fantastic responsibility. The other teachers had been to Teacher Training, I was with another VSO, a really nice woman who was teaching biology, she's now become a GP and so forth, and we shared a flat together, but all the other teachers were, obviously, Jamaican teachers… Madeleine was teaching biology, I was doing geography and maths. But the point is that we had to do the whole of the 'O' Level syllabus – which is a two-year syllabus – in one year, because the students hadn't done anything. So I started maths, "What do you know?" They hadn't done logs – logarithms – and in that day and age you did logs in the third year of secondary school and the 'O' Levels were in the fifth year, so you had to do more than two years of education of maths in a year. Well, we survived. We did it._We had to do it.

They didn't have any maths 'O' Level books. So the first weekend, Madeleine and I, once we realised that, went into Kingston, two hours on the bus basically, to buy a set of 30 maths books. And then I was teaching geography 'O' Level, they had no maps! In geography 'O' Level, you had to do map skills, you had to do Ordnance Surveys. So another trip into Kingston, buy up a whole load of maps of Jamaica. 'O' Level geography was about the geography of the Caribbean and Jamaica, so I had to read up all about bauxite and tourism, and the geography of those areas, coral atolls and all that sort of thing. So it was incredibly

hard work, and yet every evening you were preparing for the next, you were reading up what you needed to do to prepare for the next lesson. So I think we were very proud that a number of the students passed, I think – I can't remember – I think it was about 8 or something passed 'O' Level maths. These were kids, you'd see them on Saturday in the market, bare foot, riding donkeys, because they were smallholders, and they'd be going down to the market, in shabby clothes and no shoes Some of these kids did go, and some of the others who got 'O' Levels, getting two 'O' Levels was enough to go to Teacher Training College.

But it was so cut off, for example, that there were no telephones. If you wanted to try and telephone, you had to go to the Post Office, and you could sometimes book a call, but normally we'd communicate with the other VSOs in Kingston by telegram, so it was the only means… I didn't speak to my parents for over a year. So in terms of independence and so on, it was incredibly formative…

I got to LSE and my friend, Bridget, was doing geography at Cambridge, and I thought, "What do I want to do sociology and social anthropology for?" And I tried to change to geography on my first day there, and they wouldn't let me, which really annoyed me. But anyway, so I had to start the social anthropology pathway. And then after about eight weeks we'd seen all these films of people dancing around in grass skirts – and read about whatever, tribes here, there, and there, and getting a very close knowledge about these tribes. I thought, "Why do I want to know about these tribes in New Guinea, or Africa?" Evans-Pritchard and so on. "It'd be better to have an in-depth knowledge about my own society". So after about six weeks, I changed from the social anthropology pathway to the sociology pathway.

But then I was in the second year at LSE, and I got a bit bored, and I thought, "I want to go back to Jamaica." So how did I make that work? I applied to the University of the West Indies, in Jamaica, to go as a one-year "special student", so-called "special student", or "occasional student", to do a whole load of courses at the University of the West Indies, in Caribbean Social Thought, and Caribbean Economic Policy, and Political Thought in the Caribbean. I was taught by really radical great people, and economic problems and capitalism and so on, I remember a guy called Girvan, a very very radical economist. This was the time, I was 19… The rest of the students were Jamaican, apart from there was one Swedish student who gave me this ring, because he'd bought it to give a girlfriend, and then broken up. He wasn't a boyfriend or anything, we were just the only two whities, basically, there! …

Actually I started living in a maid's quarters, and one of the most horrific nights of my life was the first night in this maid's quarters in the garden, back garden of a middle-class home – just me, only cold water, little cold shower. Went to bed, and suddenly woke up, and there

were cockroaches everywhere! I mean, thousands and thousands of cockroaches. And I couldn't do anything. I was imprisoned in this little box in the garden… It was the most horrific night of my life. But anyway, the next day I went and got a few cans of cockroach spray, and at least got rid of that. So I had one term, in that maid's quarter, then moved into somewhere a bit better. But so I self-funded that by doing a couple of part-time jobs at the hospital, research type jobs... and that gave me enough money to live on while I was in Jamaica. (pp. 9-14)

3: To Medical Sociology

I went back, but it could have been the final, but because I was doing my final year. Then I knew I'd got this Medical Research Council Scholarship to do the Masters in sociology, as applied to medicine, at Bedford College at the University of London. And that, that was formative. It wasn't that I particularly wanted to do medical sociology, I applied for various things. I applied for jobs in the police – they rejected me! I applied to Teachers Training College, they rejected me, because I was doing sociology. So, I applied for everything. You know, what comes out, comes out. There was no game plan in any way, shape or form! (LAUGHS) So I got a Scholarship to do medical sociology and I'm a medical sociologist – you know, 40+ years on.. – I mean, and I still know people who were on that course with me. It was the Sociology in Health and Illness, so Sociology and Medicine. So, yes, I was interested in it. I did my Master's dissertation on class and inequalities in health. I remember analysing, I wanted to get some data from Scotland, which was computer print-out of people who are in hospital, and it had various characteristics of people and so on, including their class, and I don't remember how I did it, but somehow I analysed all these print-outs about class inequalities in health or hospital. But, I continued to be interested, and still am. You know, that was a thread running through my career, class, inequalities in health. So I was obviously interested in inequalities then. (p. 16)

4: Ann Arbor

I think having been to America, travelling around, yes, I had that boyfriend at one point and so on, there, and, but it wasn't anything particular. I don't know, I think it was just a land of

opportunity. So I think I wanted something different. I think what characterises me is actually some different experiences, different things. So I could have gone to America to do criminology if I'd got a Fellowship, or demography – I got the demography Scholarship, so that's how I ended up in this - which was an excellent, excellent place – at the Population Studies Centre.

So I did high level stats courses, and regression, sampling theory, lots of methodology courses because they were very strong on that – demography, yes, courses – method and demography. And I also did some health courses, having done medical sociology, and they were very interesting too. And I did courses like ethnicity and race, because I was interested in ethnicity and race. So I did a wide range of courses there. And I did extremely well, on the scales and so on, and I could have gone on to do a Ph.D. there, and all my contemporaries did, and have become professors in America.

But I got bored with Ann Arbor, because Ann Arbor's nice, it's a university town, a very middle-class town, and it's 40 miles from Detroit, but it's like a million miles away. So Detroit is inequality and poverty, and black, and all that. Ann Arbor's a white, middle-class university town, and there's nothing to do there. If you go to the theatre, everybody else is going to the theatre. And it's very cold in the winter too. I got bored. So then I think came about the spring time, or May time, or something, April, I then applied for jobs in England.

We did all our work assignments – and this was at the forefront, this was cutting edge stuff at the time, 1974, 1973/'74 – so we were using punch-cards, we were analysing real data, so general social survey data and other US, so the US was way way ahead of us in terms of methodology. They had what they called "Public Use Samples" from the American Census, from 1950, 1960, 1970, and so a lot of my colleagues, both the lecturers and the academics, and the students, were using real data.

While I was there, the formative influence was a woman called Karen Oppenheimer-Mason, who was a lecturer. She was probably only about eight years older than me, but she was a lecturer at Michigan, and she was doing analysis of data. When we were at Michigan, we all had to be what was called a "research apprentice", work for 12 hours a week for an academic. So I was working on one of her projects, something about gender and attitudes, and family, and all that sort of thing, and we ended up writing an article – mainly written by Karen Mason! I think my first draft was absolutely terrible! But anyway, so she was very good at helping me write, because I was doing the analysis, and doing first drafts, so that was actually published in the *American Sociological Review*, the prime journal, in 1976. So that was my first publication, and probably my best publication! (pp. 17-20)

5: SPSS at Ann Arbor and Surrey

Basically it was just a computer programme which, to analyse data files. You do a survey of a thousand people, and they're all asked lots of questions, and that data file has got rows of variables, answers to each of those questions, probably columns, and then the other way round would be the [answers] of each person, so you've got, like, a big matrix there, of a thousand people, lines of their data for each of those questions. So it's just relating the different things together, so you could look at, "What is the level of class inequalities in health?" You know, "These are the health of people who are middle-class", "These are the health of the people who are working-class. Are they different?" "Why are they different?" And then putting in other variables to try to understand why.

The data would have been on magnetic tape, at the time, so the data would have been put on punch cards, and then the punch cards would have been read on to a magnetic tape as a big file. Read by a computer – big main frame computers then. And then the programme was SPSS Programme. We would write, so I learnt how to write SPSS syntax – that's the only way you do it - so you could re-code age as a linear variable into age groups, or education or what have you, and then do a cross tab of health by class, by age, so you're looking at class difference in health, and does that differ by gender, or differ by age and so on. So what I learnt there was to do SPSS runs, to do the statistical analysis. So I did the statistical analysis courses, which wasn't just cross tabs, but it was kind of multiple regression and other sorts of function analysis and this, that, and the other. But we would do the exercises using SPSS to do that statistical analysis. Then we'd have to interpret the statistical analysis which came out of the SPSS – the print-out. So we would do the cards on a punch card machine, you'd sit at a machine here, and you'd punch like typing, and it types patterns of holes, basically, and those patterns of holes are read by the computer to mean, you know, whatever the instructions are, and it's sent away to the computer, and you get your results the next day, or an hour later or something, and that comes out, it's printed out on this great computer paper.

Cross- tabs are just the relationship between two variables, that's all. Yes, just simple things. [Before SPSS] we did it all on card sorters and counter sorters. I remember visiting a woman called Ann Cartwright, who did surveys early on in the late sixties/early seventies, and she had a little survey organisation, and they had these counter sorters. So you'd do the cross tabs, and you'd do basic things by putting these cards into a big machine which would sort the

number of people who were working-class, good health, who were a certain age, and then sort them into piles – it's almost like a kind of counting machine… But before that, people did it all by hand, and that's what I did for my Master's. I was looking at rows of columns of figures, and rows of things, and trying to relate them together manually.

So when I first came here in 1974, not long after I got here, the Head of Department was very proud that we had a room with calculators in, and these calculators were 12" square, or bigger! And they were just doing what a basic calculator would do, and this was the first time we had calculators, so that students could calculate things – rather than do it by hand – on a calculator. So when I was asked to teach research methods and analysing data, I'd get the students to do SPSS, I taught them SPSS basically, and they had to punch their cards to do a little SPSS, something like do a re-code, and then do a cross tab – one variable by another variable – and then control on a third variable, quite simple.

We didn't have a proper computer here, we had a Computer Centre, but the computing was done at the University of London Computer Centre, so that the cards somehow would get sent from our Computer Centre to the University of London, and then the next day you'd get the results back. So if a student made a mistake on their syntax, one little mistake, then it would come back as rubbish, didn't work, and then the next day they'd have to re-punch the cards, and then re-send them back up to London, and then they'd get them back again. So if they were doing course work using SPSS, it could be quite a lengthy process because of that, and if we were doing analysis that was the same thing. That's how we did analysis – via the University of London Computer Centre. (pp. 20-3)

6: The Pleasure in Numbers

I like numbers, yes, yes, yes. And that's why I'm a quantitative sociologist I suppose. But I like numbers because of what they're trying to tell me about class inequalities in health, or gender and inequalities, or whatever it is. You're analysing somebody else's data, yes. You don't need - you just need a data set. You don't need to talk to people. (p. 23)

7: Teaching Quantitative Analysis and Finding Data: the General Household Study

I needed to use the UK data because I was employed here to teach research methods. That was my key thing, that was to teach research, particularly quantitative research methods, which was analysing data using SPSS, real data, yes, looking at causal analysis of various kinds, and then it was looking around – so I think we began by using American data, the General Social Survey, which was available and I could get hold of.

But then I fished around for, you know, are there any UK data sets? And at the time, there weren't. But meanwhile, I was also interested in continuing to pursue class inequalities in health, which had been my Master's dissertation, and I found the General Household Survey Report, which had been published, based on the 1971/1972 General Household Survey, which was their first big government survey, general survey, collecting all sorts of facts, and it was really, and I read it incredibly thoroughly… I thought, "Well, it's a gold mine of data", because it was a sample of, I don't know, 10,000 or 12,000, some large number, and they collected all of the good information about the characteristics of the individual, their socio-economic circumstances in all sorts of ways, their family circumstances, lots of questions about health, lots of questions about use of health services, lots of questions about chronic illness, and all sorts of other things. And I thought, "Well, they've analysed it in their Report, but actually the theoretical questions, or the policy questions I want to address, they haven't analysed." So I thought, I was interested particularly in how need related to use, and so on, I had some kind of concept about it. Because there's no point in looking at use of health care if you don't know what the need is, so you've got to relate what is the health status of people and their levels of chronic illness and so on, to their use…

Well, it was one of those surveys which was commissioned by all the different Departments of government, so the housing people, Housing Departments, wanted some questions about housing – about house ownership and how long people had lived there, whether they wanted to move. The Health Department wanted questions on health. There may be another Department wanted questions on employment and why people are changing jobs, or unemployment and so on. So it was one of those omnibus type, broad surveys, with different sections on different things. So the analyses the people at what was then OPCS – the Office of Population Census and Surveys – did, were done for the government Departments. So they created variables, lots of variables themselves, but they created variables which were useful for the analyses that the home Departments wanted them to do. So they had loads of ones about different bedroom indexes, I seem to remember, because you can relate the number of bedrooms in a house to the number of people and the age distribution of people, so they'd got loads of questions, things about overcrowding and housing, because at the time that's what

they were interested in. But they hadn't created some of the derived variables that I would have been interested in. So you had to get hold of the raw data to actually do the analysis I wanted to do.

Because I was really interested, I went to talk to people. There was one called Mary Durant, worked with OPCS, worked on the General Household Survey, and I remember going to meet her and talk to her, and I said, "I'm really interested in your stuff", "I'd really like to do it", and she said, "Oh, we don't give the data to anybody". "Oh", I said, "Can you do the analysis?" "No, can't do that there". So I talked to various people, and then she said, "Well, the only way is to write to some senior person there, and see whether they would okay it". So I wrote off, and then I do remember having this audience with about eight people from OPCS, including this guy called Abe Adelstein, who became the Chief Medical Officer… So anyway, I had this great audience with various people there, about, "Why do I want it? What was I going to do with it? Would it be safe/secure?" Nobody before had ever used government data. It was collected by the government for the government, with government money, why would they give it to some unknown 25 year old, 26 year old woman, working at a university? It was unheard of. No precedent for it.

But they could have thought how much use it could, that could be used -

Well, that's later on. That's how it got through, and how the Data Archive later on, with Howard Newby, negotiated with all the government Departments to get all the government data sets. That was probably in the early eighties. But you had to show that they were there and they could be used. So anyway, we had this audience and I must have reassured them, and they eventually said, "Well, we could send you the tapes". I think this must have been early '75, they could send me the tapes for '73 and '74. (pp. 24-7)

8: Deciphering the General Household Survey Tapes

I got them to print out what was on them, at the Computer Unit, and the file structure was incomprehensible. It was all alpha numerics, and because it was a Household Survey, they interviewed all different numbers of people, and it was just like this block of data, with each person in the household kind of listed, but no indication of which person it was… They had a

whole load of information about the Head of Household, their job, and their age, and their health and this, that and the other. And then a whole load of information about the second person they interviewed in the household, which could have been a child, or a son, or somebody else living there, and then a third, fourth and fifth. So they'd have all this information about each person, but you couldn't really easily distinguish who the people were and what the variables were. I mean, it was just impossible… So then I applied to the University to get a small grant, which I think I got something like £400, it wasn't a lot, to get somebody to programme it, and to make it readable, and I couldn't find anybody who could do it.

Then along came good old Nigel Gilbert! Yes! Who was appointed here in September, 1976, as a first lectureship, and he was a computer ace. He'd done engineering as a degree at Cambridge, then he did a Ph.D., moved into sociology, did a Ph.D. - *The Sociology of Science*, with a Ph.D., but his father… was a Professor of Biology at Birmingham, and his mother was highly educated, so he came from a very different background. But I remember him saying he'd programmed the timetable for his school, on a computer, so he did computer programming for his father basically, in his teenage school years. So he went to university knowing all about FORTRAN or whatever programming they did then, and so he came here. So when I had this tape which I couldn't find anybody in the University, nobody in the Computer Department or anything like that, to pay money to read it, he then managed to do it...

Then when they could read it, it was in a very problematic format, so partly was kind of ensuring whether the right people in the right household, and who's who, and it had no - so he could read it, but then it didn't have, like I'd be used to SPSS files, which had variable names, variable labels, had missing values assigned, had all the things you'd expect, like I was used to, working, coming from Michigan, I'd learnt how to do. A good data file is properly cleaned and managed, and you've got variable labels, value labels, missing data signs…

Then having created some SPSS files, we then – which you could do fairly easily, you could assign people to the household, because the way it was structured, you had household information and then information about each person - so you could add the household data to each person, so you could create a flat file… But the challenging thing for Nigel was, you didn't know who was married to who... I was interested, how did the husband's information, like the husband's occupation and class, influence the women's health? So what you couldn't do with these flat files, which we flattened, was link partners to each other. So you could give the husband information about the wife, and give the wife information about their

husband…

"Flat" means a square, rectangular – a rectangular flat file. So SPSS only works on analysing flat, rectangular, square files…It's a flat file. It's a row of columns of, of variables. Not multi-level. A row of variables on one side, and then a row of cases on the other side, so you've got to convert whatever file structure into a flat file. You could reasonably easily convert, get the household information and add it to each person's record, and then you've got a flat file, rectangular file, of people, with each person having household data attached to them. But what you couldn't do is link the people together – the household members, together.

Nigel first wrote this programme, very skilled programmer, called "The cross-linker", which cross-linked people, so that pre-dated "SIR". And then SIR was something called "Scientific Information Retrieval", which the government offices started then using with the GHS, probably in the late seventies, around that time, for their managing the hierarchical data. So well, the cross-linker did something, but actually SIR did it far better, because it was a proper programme… We were able to then link people with this cross-linker, but then when SIR came along, you could do that far better.

That's when Angela Dale, who was married to Jeremy Dale, who was a lecturer in microbiology, lived in Guildford, had two children, was a housewife, but I think before that she'd been possibly a maths teacher, but she came and did our part-time Master's in Social Research, as a housewife – as a lot of these women did then, trying to re-route herself back into employment. She was obviously very able, so she was the person, Angela Dale, who worked on this project and did the SIR and so on and so forth, and then moved on to very significant things in British sociology, and social science…

So out of all of this comes the book, Doing Secondary Analysis. *Why hadn't we been doing secondary analysis here?*

I think all sorts of reasons. I think Britain was just way behind America in analysing data and having accessible data to analyse. And the tradition in sociology hadn't, in the UK, been an empirical, quantitative sociology tradition. Whereas in America, since after the Second World War, surveys and analysing data had been very much the tradition, and they'd collected lots of survey data and had access to census data from then… There wasn't a research tradition, it was more theoretical. You know, there was obviously some qualitative work, good qualitative work going on. But there wasn't an empirical research [tradition]. (pp. 27-

32)

We used them initially, so as soon as I got my first flat file of General Household Survey data, probably in 1976, the students were using it here. So we created a small data set of a thousand cases, maybe it was 1975, or 1976 probably, and they were analysing it. And then we thought, "Well, there's a market out there, or other people", we'd give talks at various conferences and meetings about teaching sociology, and say what we did, and they'd say, "Oh! Can we get hold of your data set?" So we found we were sending them to people anyway, so then we thought we'd systematise it, and actually got grants to do it as well. (p. 35)

9: Mixing Methods at Surrey

Asher Tropp's view was that [at Surrey] all researchers, all staff, should be doing empirical research. All students should learn how to do empirical research, it doesn't matter if it's qualitative or quantitative, but they should be collecting data, and analysing data about real-life issues, not just sitting and doing armchair theorising. So we all did that. So that's why I think we were very vibrant. We [also] appointed in qualitative research…
I think that it has influenced British sociology…Anthropology and Sociology were completely separate. Well, if you were doing theory, you were doing Durkheim, Weber and Marx, and very close reading of those people, and then a few other theorists. Iin the sixties there was a lot of good empirical research about deviancy, the Chicago School, actual bad gangs, and beat hustlers, and a lot of it was good qualitative research about criminal subcultures. Probably more qualitative than quantitative, in many ways.

And did you have respect for both?

SA: Oh yes, yes, yes. I think this is what has been fundamentally important about the Department of Sociology at Surrey, is that we are a Department which equally respects that there's a place for all these methods. I've taught qualitative methods – not very well – but on undergraduate courses, and if you're going to do good quantitative research, and I've run some surveys, you've got to start that good quantitative research with qualitative research. How do you know how to ask people things? What to ask people? So that good qualitative

research underpins a good survey, and to ensure that you design good survey questions, you've got to do some qualitative research…

But we were teaching it years ago - to make sure those questions actually mean what you want them to mean. Rather than abstracted empiricism. So again, when I was a graduate student, we all read Cicourel, and we all criticised abstracted empiricism. Bad surveys are bad, but it doesn't mean all surveys are bad. You've got to get good surveys, and you get good surveys by doing good qualitative research to design them, good pilot work. (pp. 33-4)

10: Sampling the Census: Ensuring Confidentiality

I probably pushed it forward more because I'd been in America, so I knew the value of the public use samples of US Census data, because all my Ph.D. colleagues and academics who were working there were doing fantastic stuff about racial segregation and so on, which in America was obviously very important, using American Census data. So I knew that, and so I was very keen, right from Day 1. I was on the SRC and a member of the ESRC Working Group on the 1991 Census… So I said, "No, we've got to try and do it". So then Cathy Marshall obviously was closely involved as well. I don't know that she'd had that much experience with it, but she obviously spearheaded it, and realised the importance of it - no question…

Well, we wanted a sample. You could get one per cent, or five per cent… So it was eventually agreed that the Census Office wouldn't give us retrospective samples from Censuses, but they'd make the 1991 Census available as a Sample of Anonymised Records [SAR: sample for use by the academic community]. And I must admit, I've toyed with [using it] a couple of times, but I've never properly used it. There are a number of [possible] different samples – 1%, 5% - or you can get samples of aggregate data based on wards, samples of individual data,. so if you've got a 1% sample, it'll just be a random 1% sample of everybody… But if there is anything which is potentially confidential, the top [coding] people – [because it's so obvious who they must be], you can't have somebody who's 101, so anybody over the age of 95 or something, or 90, is put in a group which is larger, because [otherwise] you could be identifiable. So there are all sorts of ways of trying to ensure confidentiality. Because that was the big fear of the Census Offices, that somebody would get identified. As you know, if you identify them then you can find out more about their other characteristics. So there was paranoia about that. And there were various ways of grouping

the values on variables, to ensure that you couldn't identify somebody with more than eight children, or somebody who was over 95, or what have you.

It's much easier to be conservative and not give it out, but because there was this big groundswell of academic opinion and the survey which Cathy Marshall organised, and various other people did, professional bodies and academics saying, "This is how we could use it, which could be a benefit of society policy", then it was hard. Then the only objection of the Census Office was breaching confidentiality. That's when Chris Skinner and various other people did these simulations about the likelihood of a breach of confidentiality which was published in *Journal of the Royal Statistical Society*. So, it was the other people who do it, who could then say the categorical chance of this is infinitesimally small, particularly if we do this kind of top sizing. (pp. 35-7)

11: The Relevance of Women's Occupations, and the Professional Stigma of Being a Mother

I started with a Masters in Class, Inequalities in Health, and at the time, people just analysed men, or they analysed women in terms of their husband's occupation. So women's health was classified in terms of the occupation of the husband or head of household, and all of the figures and analyses and General Household Survey published reports were according to head of household. So the argument is, by the late seventies/early eighties, this is no longer appropriate… So you could look at households, sometimes the woman's got a higher occupation than her husband – normally it's the husband has the higher occupation – so cross-classification of them, or to what extent it is helpful to look at both occupations. I did various analyses to see what difference it made. Obviously one of the problems [was that] in the early eighties, most women were not working, or not working full-time. So the majority were working part-time, but they were in jobs which were below their job which they had before they gave up work to have children – because that's what women did. So then it was looking at that kind of trajectory – should we be looking at women's occupation before they had children, or women's current occupation, knowing about occupational downgrading. So there was a lot of work to be done looking at women's occupational careers and women's part-time work, full-time work, not working. And how do you classify occupations for women if they're a housewife? Do you look at their last occupation, when was it, or do you then substitute. So it was almost methodological work as well as substantively…

Because at that time, it was the head of household, the father's occupation, which determined the living standard of the whole household, and whether you could get a mortgage, everything to do with the lifestyle of the housing, when you've got the tradition of the housewife in the homes from post-War Britain through till the early sixties or late sixties or later, was to do with the husband's occupation. The wife was completely irrelevant…
Exactly. So that was the time when you got Bowlby and all this stuff about how children are disadvantaged by their mothers working, and the key thing for a child's development is the close bonding forever with the mother. So there was this debate which I was part of, about whether employment for women was a source of role strain, or role enrichment. We obviously came down on the side that actually, employment is generally good, but recognising that that's class, and it still is, class divided. So it might be a source of strain if your husband doesn't help at all with the housework, if you haven't got any other support, and you're on a low income, then actually working full-time may be quite a strain for the women. Whereas if you're a woman who's working full-time and you've got a comfortable existence, your husband helps, you've got money to pay for cleaning, a bit of help, and you've got a flexible job, and you can cope with it, it's enrichment. To some extent, some of those things are still with us. But at the time, it was not revolutionary, but it was an important theoretical issue. (pp. 38-40)

I started to lecture at 25 and I had my first child in 1981, when I was 32, and then the second when I was 34, and my third when I was 36, so in the eighties I did have young children. I was one of these strange mothers – at the time, you couldn't admit you'd got children – in the sense that I didn't take any maternity leave, a month or two maybe, if that, because if you were going to be a successful career, you couldn't - I didn't have any pictures of my children up or anything like that, because that would be seen as a kind of negative black mark, and you couldn't suddenly leave and say, “I'm going to pick up my children”. That's terrible, but now, obviously, men do that, and they get Brownie points for it! But it was very different in the eighties, a very different image. (pp. 40-1)

I think there are always the difficulties of time and energy, and if you've got a full-time academic lectureship, and a number of children, it's a kind of time, but that's the advantage of secondary analysis, you can do it at midnight or whenever. You can do it whenever, weekends and so on. (p. 42)

12: The Dangers of Good Publicity

It was a survey looking at changes in General Practice in the 1970s, and that was a time when people were moving from solo doctors to Health Centres, larger group practices, appointment systems, receptionists and so on. So I designed and ran the whole of a survey of several thousand people in Surrey – a representative sample of people – about their attitudes towards general practice and services in the Health. I wrote various things up from that, several published in the *British Medical Journal*. I produced things about the receptionists, and how a lot of people felt that the receptionist was a barrier, and that we'd got these people saying, you want to make an appointment, you can't get through the receptionist because they ask what the reason is, and they diagnose this.

I gave a short presentation, I think it was at a Community Health Council meeting, about this research, and there was a reporter there, who then reported it in the *Surrey Advertiser*, and then it got about "The dragon behind the desk". It got taken up by all the newspapers, and I was *Daily Mail* banner headline across page three, or five, about this "dragon behind the desk". I was actually invited on to *You and Yours*, to talk about – and of course, they always pick the two people that are opposites – so I was on, and then there was the Head of the Medical Receptionists' Association, and so there's me trying to defend it and the role of the receptionist, etc., etc., and so more publicity!

Then various receptionists wrote in to me and said, "You don't know what it's like". You know, "We've got all these patients who come in - der, der, der". And I said, "Yes, I'll be very happy to come and talk to you and find out what it's like". So on the basis of that, I went to probably a dozen different practices, after we'd done this survey, to talk to the receptionists, and spend a couple of hours in their waiting rooms and see what was going on. That was kind of, yes, pseudo qualitative data, so again, I could use that in publications as well.

But also I got into real hot water with the Department of Health, because they funded the research, and I had to clear everything with them before it went into the public domain, and, of course, I didn't realise – I was a naive, late twenties year old, it was late seventies - and it was as I said, in the local paper, and then went round all of the nationals, and all the nationals were then ringing up the Department of Health about this piece of research, and they didn't know anything about it, you see! So I got into very hot water, and got slapped on the wrist a few times. (p. 43-4)

13: Not for Policy Impact Then

You do research because you find it interesting and important, but then it's up to somebody else to do the policy. At that time we weren't in the era of impact and follow-through. So I must admit I was interested in contributing to the academic debate about these things, and also some of the methodological debate about how best to measure the class of women and so on, so both the substantive - and you present it at conferences, you publish it, and then you go on to something else. I mean, the work's going to get quoted, it was reprinted in various places, so it obviously had some kind of impact.

Does it bother you if it doesn't, then?

No. I mean, you're not, you're not doing it for that reason. (p. 44-5)

14: The Sociology of Ageing: A Poor Relation

I mean, no, as with everybody in sociology, ageing is irrelevant. It was very much a kind of poor relation, so to speak. Sociologists weren't interested in ageing in the UK. They had been in America, and I think that's a difference with America. But in the UK, sociologists had not been particularly interested in ageing, and they still aren't particularly interested in ageing, and that was one of the reasons for this [ESRC Ageing] Initiative. So I was interested in health, and also employment, because I was interested in women's employment, so that when this ageing initiative came along, we thought, "Well, better apply for it!" "What can we do?" And that started it. So it was older people and health, older people using health services. Older people need health services because they need care, and they've got disability. So then we need to look at all those things that older people – their health, their disability – who provides, what sort of statutory services they receive. Do they receive informal caring?
So it was really the initial looking at informal caring. At the time I remember writing an article, and those were the days when we had a secretary to type them, I'd written something out, and she kept writing "career" rather than "carer", and that's because the word carer hadn't been invented. It didn't exist in the lexicon, so we were the first people who started writing about "carers", and "informal carers", in the mid-eighties. So that's how that moved along.
So then I, yes, got really interested in ageing. "Why isn't there much more sociology of

ageing, and where does gender come in?" Because a lot of the work on ageing was very much about policy and about older people as disabled, frail, needing care, needing services. It wasn't treating them as normal people, sociologically interesting people. It was treating them as the "those who", they were the subjects of policy, so to speak. So then I was using more the sociology of ageing, and how women all fit into that, and gender.

I thought it was interesting that the Feminist sociologists weren't interested either, because it was elderly women.

Exactly! No, no, exactly! They weren't, because the Feminist sociologists were interested in children, child-bearing, employment, the way in which children impact on employment, and reproductive things. I mean, at the time I taught courses on gender and society here, at Surrey. My course on gender and society was all about young women's issues, obviously contraception, blah, blah. Mid-life women, women and children and so on. But then, once you get to the menopause, somebody mentions the menopause, and nobody's interested in it at all. And I'm not saying that's the case now, but there is still something of that, Feminist sociologists have perhaps less interest in older women. But that has changed. That was another thread of the work, and I think that has been influential, that we've brought onto the sociological agenda, issues of older women, and inequalities in ageing, and gender inequalities and class inequalities, and other sorts of inequalities of ageing. (pp. 47-9)

15: Gender in Later Life

Well, *Gender in Later Life*, if I'm looking at the significant work in my life, that's probably one of the most significant. People still quote it, and saw it as a gender setting and really changing the mould, because there hadn't really been any book-length treatments of gender and ageing, gender in later in life, at all – anywhere – and certainly not from a more sociological perspective. So it was a sociology, and it was broadly based. It used quantitative data, looking at older women's disadvantage in terms of their pensions particularly, old age disadvantage in terms of levels of disability, old women's disadvantage of living alone – widowhood and so on – older women, and access to care givers and the care giving and so on and so forth. So it went through a whole range of things. And it really did have a big impact, both in the UK and internationally, and that led to invitations abroad to do

things. And people are still quoting it. It was totally quantitative. It was based on analysis of General Household Survey data.

I think it had an impact in the sense that it put onto the agenda Feminist sociology of ageing, older women's issues, from a sociological perspective. That you can't just talk about older women, older people as all deprived, disadvantaged and so on. I was looking at what amongst older people is a range. Some men are quite advantaged, most women were not terribly advantaged - and why they weren't advantaged? I think it was bringing out disadvantage and advantage, and what are the mechanisms through which those advantages/disadvantages become perpetuated.

Then, obviously, a whole load of work looking at pensions. So women's pensions being absolutely terrible. Divorced women - divorce had been increasing in the seventies, so older women entering later life as divorced were completely disadvantaged. They were the poorest group of any, basically, whereas widowed women weren't as disadvantaged as married. But a lot of – not all – but a lot of women, widowed women, had inherited part of their pensions, because women didn't have pensions in their own right. So part of all that work on pensions was to what extent women do or should have pensions in their own right from their own occupations. (p. 49-50)

I'm not saying things aren't as bad as they were 20 years ago now, but it's still the case, women tend to be disadvantaged, financially, in later life, quite a lot, and they're much more likely to be disabled, and they're much more likely to be widowed and living alone, etc., etc., but they do have better friendships and networks.

You see, men die from things, and women get disabled. But now more men are surviving with disabilities as well, because diseases used to kill people, now they kind of carry on. So there are much more longer-term marriages than there used to be – the gender gap in mortality and in the expectation of life is now less than four years, used to be about seven years, and obviously they're then compounded by the age differences in marriage. So women are still much more likely to be widowed, so that, as I was saying, men die married! If you look at men over the age of 65, about 85% are married, and then if men are widowed, they're much more likely to remarry than women. (p. 50-1)

And now you're older, do you see it all kind of coming true?

No, no, because I've been one of the very very few fortunate women who had a full-time

career throughout my life, a full-time, good career at a university, with good benefits and pension, so, I mean, I'm rare. The vast majority of older women and my contemporaries, are not in that position. (p. 51)

The importance of ageing and gender, I think that's been a thread throughout my career. Yes, I'm an ageing researcher, but I am somebody who looks at gender. [We don't] just look at women being disadvantaged, we've done projects on older men, and looked at to what extent certain sub-groups of older men, particularly divorced older men, unmarried men, are really very badly disadvantaged.

Is that because they don't make friends?

Yes, but that's not their fault! We can't say, "You don't make friends?" That's true. But it comes back to socialisation, upbringing, cultural expectations. (p. 51) And being non-normative: there are lots of widowed women out there, and if you go to U3A, and you go to all these things, yes, you get women, lots of women, and then you get couples, but there aren't many men alone in lots of these organisations. There isn't places for them, spaces for them. So you can't just blame that they don't make friends.

They're more likely to lack social support in various ways, and particularly divorced and never married men, don't have the same - they don't have children, or they don't have a good relationship with their children. So in terms of care-givers, they don't have access to family care-givers in the same way that divorced women do, and never married women do. Because never married women often retain good relationships with their siblings, and their nephews, nieces, and all that sort of stuff, so they remain much more family-oriented than never married men. And divorced men often have bad relationships with their children, in traditional families.

Because if they're divorced when the kids are young, and they've gone off with somebody else, in that generation often the men didn't take a big role in childcare then. I think, now, with lone fathers now, it's really very different, but that was the generation when the fathers, once they'd got divorced, they were, there wasn't a lot of contact, or good contact. Yes, so I think when we're talking about gender, we do need to understand gender processes of various kinds. (p. 52-4)

16: Against Women's Studies

I'm a Feminist sociologist who is interested in gender. But I've always said I'm not interested just in women, and when I've taught courses here, I taught a course on Gender and Society, I didn't teach a course on Women and Society, and I wouldn't support courses on women's studies – I'll support courses in gender studies, but not women's studies… Yes, it needs to be comparative, absolutely. Absolutely, yes, yes. So you can't just look at women, you've got to look at men and women, and actually how things differ and why things differ – analytically. (p. 54)

17: The Sociology of Sleep

A colleague in the University, Dr. Debra Skene, who's in physiology, kind of biology, she happened to come and see me in something like 2000, and said they were putting in a big EU project, with women and scientists from different European countries, and they were all medical scientists, physical scientists, and they thought it would improve their chances with the European Union if they had a social scientist involved, or sociologist involved, and might I be interested? And the project had already been designed from the science side on women and menopause, and sleep.

So I thought, "Sounds interesting". I didn't know any sociologists doing any work on sleep, or work on women and menopause and sleep. So I designed some work packages on the sociological side of women and menopause and sleep, and I think on the second time we put the application in we got the grant. So basically we started that joint interdisciplinary project, and myself with Jenny Hislop, who was the researcher on the project, were involved in all the meetings, and we did a separate aspect of research on sleep, which involved multi-methods. Because nobody knew anything about sleep, so you can't start doing quantitative data, there was no quantitative data on sleep.

So we designed… ten focus groups. Different age groups of women. We just got them anywhere. They were from groups of people, Jenny Hislop recruited them, they weren't particularly representative… There were about eight in each focus group. So there wa about 80 women, different age groups.

Then because we didn't know much about sleep, before we did the focus groups, we trialled what we called "audio sleep diaries" on ourselves. So we thought how do we know about

sleep? It's hard at four o'clock in the afternoon to remember much about sleep the previous night. The best way is to use a little audio recorder or dictaphone, and when you wake up in the morning or whenever, record a few things about how was the night: what time did you go to bed, did you wake up during the night? If you woke up, what woke you up? What did you do to try and get back to sleep? Were you disturbed by other things? Etc., etc., etc. So we did that on ourselves and some other people in the Department did them as well – other women. They were incredibly insightful. [But] they didn't realise how routine their sleep was, or how much they were disturbed, and what, for them, made a good night, so we also asked them to say, was it a good night or a bad night? How would they evaluate it? So there were all sorts of insights from it, about how they were disturbed at night, impact of husband snoring, impact of children, impact of all sorts of other things – you probably know, but hadn't really been talked about. So the knowledge from the audio sleep diaries were then put into the focus groups, and basically the focus groups then gave us lots of insights about things we really hadn't thought about.

So we knew about asking about partners and snoring and things, but then, when we asked, are there any times when you sleep separately from your partner, under what circumstances, and people would say, "Only when I'm ill" or things like that. Or they'd say, "When I'm ill, or got a bad chest". Then some people would say, "Well, you know, well, actually I'd love to sleep separately from my partner because he disturbs me terribly badly". Another person says, "Well, we do", and then they looked, everybody looked embarrassed. What is the implications of that? And then somebody else said that, in another focus group, that yes, they slept separate from their partner, and they slept far better, but when the kids came back from university, they thought the marriage was splitting up because they were sleeping separately. So all this stigma about sleeping together and sleeping separately came out of the focus groups, which we really didn't know anything about basically. And how people dealt with it, and how people dealt with snoring partners. And some things which we'd really never thought about at all. So the focus groups were extremely valuable.

And then we followed that up with about 40 in-depth interviews, one-to-one, with women of different age groups, about their sleep… Talking about what we called "temporary relocation", and "permanent relocation", whether people permanently sleep separately, or under certain circumstances sleep in separate rooms, how resources impact on that. How one interviewed a couple in a one-bedroom flat, and husband disturbs wife, husband sleeps on the sofa or whatever. These sorts of things. So, again, in terms of inequalities, the nature and quality of your sleeping, your environment and your housing has a big impact on those things

as well. (pp. 56-9)

There was so little at the time, surveys which collected any kind of data on sleep, and partly because of our work, and since the early 2000s, talking about the importance of sleep, then a number of surveys are now having questions on sleep... So there's now a much greater recognition of the importance of sleep for health and well-being, and the importance of collecting data on sleep, survey data, so now it's much more possible to do secondary analysis of existing data which includes something on sleep.

Why hadn't sociologists looked at it before?

I think most people just think it's an invisible part of life. It's interesting, because sociologists – over the last 30 or so years they've looked at a lot of everyday behaviour – so sociologists look at leisure, they look at food, food within the household, but sleep is not - it's every day, every night behaviour. (p. 60-1)

It hadn't occurred to me. Exactly! Exactly. So I do feel that we have put it on the map and put it on the agenda. Now other people are seeing it is interesting, it's an interesting vein, and it not only tells you about sleep, but it tells you about gender roles and inequalities. So subsequent to the first project on women's sleep, which we concluded by saying that, actually, the menopause doesn't have much impact on sleep, but the reason why women in their late forties, and early fifties have bad sleep, is because of their stage in the life course - women, late forties/fifties, early fifties, often have teenage children at home, they get disturbed, they have partners who snore, they have parents who need caring for, often they're at a stage in their career of more managerial worries associated with work, and so there's a whole constellation of factors which leads women, at that stage, to have poor sleep. (p. 61)

18: Audio Diaries or Written Diaries

Had you tried out writing, rather than recording?

It doesn't work. It's dreadful. Here was a woman said, “I slept fairly well”, in the [audio] diary, “I went to bed about 11. I didn't sleep in my own room because by the time I got to my

bed, the husband was in full snore, so I went to sleep in the spare bed in the children's room". Then she suddenly said, "Oh, and then I woke up for a few minutes because my daughter had wet the bed, and I had to change it". But it was a good night for her! I mean, she … brought all those things in. And if you had it written, you don't get all that, and you don't get that kind of feeling from it. "John was there", you know, "I tossed and turned for an hour and a half, I was worried about my parents who are very ill, and one of them's going into hospital". So they would talk about worries and anxieties, whether to do with work or care-giving or other things, impact on sleep, as well as pain and things like that. So they might have been in bed still, but actually they were quite disturbed by it all. (pp. 59-60)

There are a whole range of factors. And the social may be direct social worries and anxieties about them. Clearly children have a big influence, and it's not just young children – because people will be talking about their adolescent and teenage children, and how they can't get them to go to bed at night. What are they doing in their rooms? They're playing music, they've got to get up in the morning. Or they're out. And they're worried about them, they've got to wait up till they come in, all those sorts of things. (p. 60)

19: Sleep and Social Class

I was very interested that people of lower education attainment sleep worse.

Well, I think it's a whole combination of things. We don't know why, but we can show that that is the case, and it may be to do with the housing circumstances. If you live in a grotty area with sirens going off, and neighbours bashing on the walls, you're not going to sleep as well as if you live in leafy Surrey in a detached house where it's quiet and calm, and you're not worried about the safety and security and burglars and zzzzzzz. So there's the direct environmental effect, which is associated with education and income. It's not that education causes it, or income, but they are associated with those things, and then associated with worries and stresses. Again lowest income, financial, so we've looked at if people can make ends meet and things like that. Those who can't, much worse sleep than those who can. And the space in the house and so on. (p. 72)

20: On Snoring

In fact, couples were rarely negotiating about sleep, because if you start talking about your partner's sleep, it's a very critical and intimate thing. You know, "That person's criticising how I sleep, or my snoring, or what I do at night, which is beyond my control", and it's almost fundamental to the relationship. So when we did the couples, the couples project was totally a qualitative project, we initially interviewed couples together by a male and a female interviewer, so we had two interviewers interviewing one couple, and they'd take it in turns to talk. We wanted gender symmetry, because if you had a woman interviewing the couple, the man might feel, "Can't say this", and if it's a man interviewing a couple, then the woman might feel, whatever. So it was quite important having that. But you'd ask them questions about things like, "Do you snore?" and the husband would say, "Oh yes, I snore", and the wife – men's snore is treated very humorously, you know, "all men snore, it's one of those things." Whereas when you ask about the wife, "Does the woman snore?" And the woman would look very sheepish and say, "Oh no", and then the man might say, "Oh, she snores!" in a very kind of negative, vitriolic way. So then we got on to looking about, what is the gendered meaning of snoring? (p. 62)

It seems the older women were less bothered by their husband's snoring than the younger ones?

Yes, I think, again, there's a kind of tolerance. What we do find is in the early stage of relationships, sleep is togetherness, and what happens together is much more important. So if somebody starts a new relationship in their forties or fifties, it's much more like the sleeping of people in their twenties. But if you've been together for 20-odd years then, if the other person comes to bed late, or doesn't, or there are some issues, or wants to sleep somewhere else, it's not quite so problematic. Whereas if you're in the first few years of a relationship, then you <u>should</u> go to bed at the same time, you <u>should</u> do this, and you <u>should</u>, you know, all those sorts of things, because it said something about that, the nature of that relationship and partnership. (p.65)

The whole point is your body is unruly, it's uncontrolled at night. If it's farting or whatever, you can't - it's so hard to do things without snoring, but how do you kind of cope with this?

And these are the backstage areas that obviously the bed partner is aware of. (p. 67)

21: Men, Women and Alarm Clocks

One of the classic – in our couple interviews, one man said, "Well" - because we asked, you know, what was the strategy, waking up to an alarm clock and things like that, and he said, "Well, I have an alarm clock, it goes off at six, and it's on 'snooze', and every five minutes it goes off again". And after the man said that, the wife said, "Oh, when that alarm clock goes off at six, I know I've got 40 minutes of agony because it keeps going off, and then I fall back to sleep and then it goes off again!" And the man was mortified! He didn't realise that the wife was disturbed by this, and found it really problematic. And the wife obviously just couldn't stand it, and found it terrible.

So then we went back on all of the interviews after about six weeks, and after the couple interview, we gave them, they had to do audio sleep diaries for a week, and they had to wear "actigraph" watches, which are these watches which measure movement and so on, and do various other kinds of sleep tests and things. Then after six weeks we went back and did individual interviews at the same time with each partner, so that the female interviewer who had been there before interviewed the wife, the male partner and male interviewer interviewed the male partner, and talked about things. And this particular couple, the man said, yes, his alarm went off at six, but then he took his clock into the sitting room, and slouched on the sofa, and gradually woke up that way. And his wife was just really happy! So, yes, but this is the whole point , is that there are practices which may have developed over time, or got more problematic, and people just don't talk about them, and they're privileging the other person's sleep – particularly women privileging men's sleep more than the other way around, over their own sleep – and it can be quite disturbing to them. (p. 64)

22: Night-time Childcare

You also found that women, even in full-time work, are still responsible for night time childcare.

Yes, young children. Absolutely. It's much more likely to be, yes… You can't generalise

entirely, but if women are working part-time, they definitely are the ones who get up. If they're both working full-time, then it varies: sometimes it's only the woman, and sometimes you're sharing it, but very rarely it'll be only the man.

But when there are teenage children, then there was much more gender symmetry, and it was about inside/outside private things. When it was the young people who were out at night, it was often the male, the father/husband, who was waiting up for the child to come home, because that was seen as their role, and he was often more disturbed by the teenage children than the wife was. So again, it's these very ingrained gendered practices, which you're getting at by looking at sleep. So the point about the sleep research is that it doesn't just tell us about sleep, it tells us about gender relationships and couple relationships, and some of those kind of things which can't be articulated, and aren't articulated, as bases of inequality. (p. 66)

23: Care Homes at Night

We studied ten Care Homes, which is quite a big project, ten Care Homes, and in each Care Home we tried get about 20 people to wear actiwatches, and they filled in – well, they didn't fill in because they weren't capable - but for a two week period, every day a researcher would come and talk to them about the night, to get a sense of the routine of their day, and when they went to bed and this, that and the other.

So you weren't using audio diaries for them?

Those who were able to do it, did, but most couldn't, and didn't. So that the vast bulk of them - and actually, they quite enjoyed it because the researcher was somebody they got to know and they could talk to. But they did wear actiwatches where feasible. Again, sometimes that was a problematic – putting them on and off, and all that…

In Care Homes you've also got the way in which the Care Home as an environment structure/organisation, has a major disruptive impact on sleep. Checking your pulse, and checking if you're alive and all that sort of thing - and when you go to bed. I mean, they spend on average 11½ hours in bed, a huge length of time, because they had to be put to bed before 8 o'clock when they'd got more staff on – staff would go off at 8 o'clock, so they've got to go to bed at seven.

The problem was, for the carers – we've done a lot on care, talking at meetings, conferences and so on - is that most of the research on Care Homes are being done during the day. People

go into a Care Home, “”Oh, it looks all very nice”. Everything is clean, and sitting in the lounge - and nobody looks at it at night. Even the Care Home regulators didn't go in at night. They're more likely to now, but that's partly because of the work we did…

We did observations, what they call “twilight observations” - six till ten, going to bed time, and six in the morning till ten in the morning – morning time and night time, as well as daytime. So you'd see very much about how the whole Home was winding down, the strategies that the staff use to get people to bed, if they were mobile anyway, before eight o'clock or by a reasonable time, and how much noise and other things were going on at night – which would be very disruptive.

But I think people don't know. It's a kind of invisible excluded, people are excluded. When you go round the Care Home looking, “Is this Care Home really suitable for my mother or father or relative?” - nobody goes at night. You look at the bedrooms, you look at the lounge, you might be there at lunchtime, but you certainly don't see the processes of getting up in the morning or going to bed at night, or what happens there in the evening.

Were the Care Homes embarrassed by the findings?

Yes… Well, in fact we ran several workshops for Care Home Managers, who were very interested, so I think it, again, changed the climate of opinion - in terms of, “We need to be concerned, not about the eight hours of the daylight when people are there in Care Homes, but the rest of the time in Care Homes”. If you're in a Care Home and it is your home, so to speak, you've got to look at the 24 hour situation, and how that works in the interests of the residents as well as the convenience of the shift patterns and the staff. (pp. 78-80)